



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ناصره احمد حسن احمد
المحافظة : القاهرة الحي / المدينة : الازبكية
رقم البطاقة : 26711150103969 رقم التليفون : 01115188478
العنوان : 3 ح الطرايبشي
تاريخ تسجيل الإستمارة : 23-09-2026
العمر (تاريخ الميلاد) : 15-11-1967 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة الأبحاث المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: ناصر احمد حسن احمد
National ID: 26711150103969



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

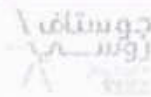
-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين





Patient's Name: Nasra Ahmed Hassam
Date: 2026-08-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Ultrasonographic examination of the neck, revealed:

- The thyroid gland is not enlarged with a homogenous, parenchymal echotexture.
 - **Right Lobe:** measuring about 1.4 cm in AP, and show small nodule. It is located in the lower pole, measuring 0.9 X 0.8 X 0.8 cm, mixed cystic and solid and isoechoic with smooth margins and no calcification.
 - **Left Lobe:** measuring about 1.7 cm in AP dimeter, with no suspicious focal lesion.
 - **Isthmus:** Unremarkable in thickness and normal in echotexture. No focal isthmic lesions.
- Bilateral parotid and submandibular glands are symmetrical, normal in size, and display homogeneous echotexture. No focal lesions, ductal dilatation, or inflammatory changes are seen.
- Large left supra clavicular heterogenous lesions, largest about 3.5 X 2 cm, and this lesions demonstrates internal vascularity.
- Bilateral small cervical lymph nodes are identified bilaterally.
- The carotid arteries and internal jugular veins are patent bilaterally.

Conclusion:

- Right lobe dominant nodule: ACR TI-RADS 2 / 3. Follow-up ultrasound is recommended.
- Pathological lesions are seen within the left supra-clavicular region, as described, likely metastatic lymph nodes, for histopathology correlation.

Best regards;

Dr. Mohammad Islam

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION AND DECISION SUPPORT



الهيئة العامة للتأمين الصحي
الإدارة المركزية لتكنولوجيا المعلومات
ودعم القرار

جهة الاستدراك: فرع القاهرة

تاريخ الإصدار: 2023-6-12

إلى من يهمه الأمر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للائتمان الخاضعين لقانون التأمين
الاجتداعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات
والإرث" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012
الأطفال دون السن المدرسية" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .
وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية
وطبقا لقاعدة البيانات المرسلة من التأمين الاجتماعي

والبحث تبين أن

السيد (ة) ناصر أحمد حسن بطاقة رقم 26711150103969

وطبقا إلى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير المستفيدة بالتأمين الصحي صدر هذا الخطاب دون أي مسؤولية على الهيئة
العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ طباعته: 2023-6-12

الموظف المختص

اسلام عبد الحميد



إفادة مركز نظم المعلومات
مستلم

مستلم 795089



MDT FORM

Patient Name	ناصره احمد حسن	Age	57
Sex	F	Special habits	
Family History		Comorbidity	HTN, DM
Surgical History		Telephone	01095172201
Menopausal Status	post	Marital Status	
ID	26711150103969	Medical ID	2112/2023
Date	9-11-2025	Reason of MDT	

Medical Report	
Pathology: IDC ER: 8/8 PR: 8/8 HER2: -ve Ki67:	MDT
Doctor's notes: 2023→ AC x4, Txt x4 Rt MRM T1N3(14/25)M0 PPR Adj PORTH, AI (2/2/2024 till now) US 9/2025 supraclavicular LNs Ex met. Carcinoma of mammarian origin ER 8/8, PR 8/8, HER2 -ve	
Recommendations :	
MDT DECISION	For:taxol weekly for 3 cycles then re evaluate

Name	حنان عزت	رائيا شافقي	احمد نجيب
Signature			

Patient Name : الأستاذة / ناسرة احمد فريحا

Patient No. : 26711150103969

Path No : 4608/23

Sex : Female

Age : 55 Y

Referring Doc :

Request No : 13005369

Request Date : 30/12/2023

Verification Date : 15/01/2024

Financial Category : أورام دار السلام - نفقة الدولة

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Clinical data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Left MRM measured 19x18x5 cm with covering skin ellipse measured 19x3 cm.
- * Tumor extent: A greyish white firm tumor bed site.
- * Size: 2.5 x2.5 x2 cm.
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 1.5 cm away from deep margin.
- * Lymph nodes number and size: 25 L.Ns up to 1.5x1.5x1cm.

Microscopic Examination:

- * Histological type: Minimal microscopic remnant of invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+) Gradell .
- Maximal tumor diameter 0.6x0.6 cm
- Actual tumor cellularity: 3%, tumor necrosis index: 97%
- * Ductal carcinoma in situ: 3%.
- * Skin involvement: Free.
- * Lymph node involvement
- * Resection Margins: Free.
- * Histologic response to therapy: Partial pathological response.

Diagnosis:

Left breast cancer post NACT, MRM specimen:

Minimal microscopic remnants of Invasive duct carcinoma.

- Maximal tumor diameter: 0.6x0.6cm

- Free Resection Margins

Verified by

Dr. Heba Hasan

Printing Date : 17/01/2024 12:12:54

Page 1 of 2

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - فورنيس النيل - مصر القديس - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Mar El Qadema - Cairo, Egypt

Patient Name : الأستاذة / فائزة أحمد حسن

Patient No. : 26711150103969

Age : 4608/23

Sex : Female

Age : 55 Y

Referring Doc :

Request No. : 13005369

Request Date : 30/12/2023

Verification Date : 15/01/2024

Financial Category : أورام دار السلام - نفقة الدولة

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Free nipple

Positive Lymph nodes (14/25)

Partial pathological response.

Stage: ypT1b, ypN3

CB: 2.907, RCB class II

Signature

Dr. Rana Fathy

Dr. Heba Hassan

Verified by :

Dr. Heba Hasan

Print Date : 17/01/2024 12:12:54

Page 2 of 2

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mistr El Qadima - Cairo, Egypt

Patient Name : الأستاذة / بامروة احمد جمن
Patient No. : 26711150103969
Path No. : 1602/23
Sex : Female
Age : 55 Y
Referring Doc. :

Request No. : 13007861
Request Date : 27/05/2023
Verification Date : 06/06/2023
Financial Category : اورام دار السلام - نفقة الدولة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Left breast mass lesion
Nature of the specimen:
Tru cut biopsy collected 25-5-2023 (1:25PM)
Gross Examination:
Four greyish white tissue cores up to 1.5 cm . Totally submitted .
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER,
HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)
Microscopic examination:
Histological type: Invasive duct carcinoma
Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
Ductal carcinoma in situ: Focal (Solid pattern)
IHC :
ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
HER-2: No membrane reaction in tumor cells (score 0)
Ki67: 25 % nuclear reaction in tumor cells

Diagnosis:
Left breast mass lesion , Tru cut biopsy:
Invasive duct carcinoma grade II
ER : Positive (8/8)
PR : Positive (8/8)
HER2 : Negative (score 0)
Ki 67: 25 % nuclear reaction in tumor cells

Verified by :
dr.esraa magdy

Printing Date : 08/06/2023 09:37:08

Page 1 of 2

Printed By : marwa mohamed yousse

Patient Name : الأستاذ / ناصرة احمد حسن	Request No. : 13009972
Patient No. : 26711150103969	Request Date : 18/09/2025
Path No. : 2863/25	Verification Date : 01/10/2025
Sex : Female	Financial Category : جوماتاف روسي - نفقة الدولة
Age : 57 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

CLINICAL DATA

Left supraclavicular lymph node enlargement.

NATURE OF SPECIMEN

Tru cut biopsy, collected on 17/9/2025.

GROSS EXAMINATION

Multiple grayish white tissue cores ranging in size from 0.3 up to 0.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

MICROSCOPIC EXAMINATION

- * Histological type: Metastatic carcinoma of breast origin
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade 2

- * IPT (Positive control showed appropriate reactivity) :
- * GATA3 : Diffuse positive
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: No nuclear reaction in tumor cells (Allred score 0/8)
- * HER2: Moderate complete membrane reaction in >10 % of tumor cells (score 2+)
- * SISH : Not amplified.

DIAGNOSIS

Left supraclavicular lymph node, Tru cut needle (TCNB) biopsy:
Metastatic carcinoma of mammary origin

Verified by :
Dr.Heba Hasan

Printing Date : 07/10/2025 09:30:11

Page 1 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / ناصرة احمد حسن
Patient No. : 26711150103969
Path No. : 2863/25
Sex : Female
Age : 57 Y
Referring Doc :

Request No. : 13009972
Request Date : 18/09/2025
Verification Date : 01/10/2025
Financial Category : جوسفان روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

IPT:

* ER: Negative (score 0/8).

* PR: Negative (score 0/8).

* HER2: Equivocal (score 2+).

* SISH : -Not amplified

-Average Her2 copy No .2.7

-HER2/CEP17 ratio 1.2

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim

Dr. Heba El-shafeey

Verified by :

Dr.Heba Hasan





Patient Name: Nasera Ahmed Hassan Ahmed	Date : 08/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID: 2015	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows UOQ area of asymmetrical density reaching the nipple measuring about 9x3.5 cm associated with skin edema corresponding by U/S to UOQ ill defined spiculated solid mass lesion located at 2 o'clock measuring about 5x4.5x2.5 cm showed anterior ductal extensions reaching the nipple and internal hypervascularity on Color Doppler.
- Right UOQ well defined isodense mass lesion corresponding by U/S to UOQ IMLN with preserved fatty hilum and average cortical thickness.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary lymph nodes are seen.
- Prominent left infra and supraclavicular lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast UOQ locally advanced malignant looking mass lesion with pathological axillary lymph nodes (BIRADS 5) for histopathological correlation.
- Benign findings right breast (BIRADS 2)

Yours truly

Dr/Asmaa Zeinhom, Msc

Signature Temp

08/06/2023 2:32 PM

Patient Name: Nasera Ahmed Hassan Ahmed	Date: 27/08/2025
Exam: US Neck	Age: 57 Year(s)
Medical ID:	Pat ID: 38846

Neck Ultrasonography

- The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echo pattern, with regular outline.
- Right thyroid lobe complex cystic and solid sub centimetric nodule (TIRADS 3)
- Normal vascularity at CCDI.
- The gland moves freely with deglutition.
- The trachea is centralized.
- Both carotid sheaths are free.
- No sizable cervical lymph nodes
- Left supraclavicular pathological LNS associated with smudging of the surrounding fat, the largest is measuring 9x6mm

Conclusions

Left supraclavicular pathological LNS for biopsy
Right thyroid lobe nodule (TIRADS 3) for follow up

Signature Dr . Nermien Ramadan

27/08/2025 12:26 PM

Patient Name: Nasera Ahmed Hassan Ahmed	Date : 08/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID: 2015	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows UOQ area of asymmetrical density reaching the nipple measuring about 9x3.5 cm associated with skin edema corresponding by U/S to UOQ ill defined spiculated solid mass lesion located at 2 o'clock measuring about 5x4.5x2.5 cm showed anterior ductal extensions reaching the nipple and internal hypervascularity on Color Doppler.
- Right UOQ well defined isodense mass lesion corresponding by U/S to UOQ IMLN with preserved fatty hilum and average cortical thickness.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary lymph nodes are seen.
- Prominent left infra and supraclavicular lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast UOQ locally advanced malignant looking mass lesion with pathological axillary lymph nodes (BIRADS 5) for histopathological correlation.
- Benign findings right breast (BIRADS 2)

Yours truly

Dr/Asmaa Zeinhom, Msc

Signature Temp

08/06/2023 2:32 PM



Patient Name: Naserah Ahmed Hassan Ahmed	Date : 10/06/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 55 Year(s)
DLP:762mGy.cm	Pat. ID: 38846 2112/2023

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of left breast cancer shows:

1. Chest:

- Left breast heterogenous parenchyma with enlarged ipsilateral axillary lymph nodes, largest measures about 1.5X 1.2 cm with illdefined enhancing sheets
- Bilateral few atelectatic bands in the middle lobe and lingula. No sizable masses, nodules, consolidation patches or cavitary lesions.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky lobulated uterine appearance is seen.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions. *

Patient's Name: Nasra Ahmed Hassam
Date: 2026-08-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Ultrasonographic examination of the neck, revealed:

- The thyroid gland is not enlarged with a homogenous, parenchymal echotexture.
 - **Right Lobe:** measuring about 1.4 cm in AP, and show small nodule. It is located in the lower pole, measuring 0.9 X 0.8 X 0.8 cm, mixed cystic and solid and isoechoic with smooth margins and no calcification.
 - **Left Lobe:** measuring about 1.7 cm in AP dimeter, with no suspicious focal lesion.
 - **Isthmus:** Unremarkable in thickness and normal in echotexture. No focal isthmus lesions.
- Bilateral parotid and submandibular glands are symmetrical, normal in size, and display homogeneus echotexture. No focal lesions, ductal dilatation, or inflammatory changes are seen.
- Large left supra claviclar heterogenous lesions, largest about 3.5 X 2 cm, and this lesions demonstrates internal vascularity.
- Bilateral small cervical lymph nodes are identified bilaterally.
- The carotid arteries and internal jugular veins are patent bilaterally.

Conclusion:

- Right lobe dominant nodule: ACR TI-RADS 2 / 3. Follow-up ultrasound is recommended.
- Pathological lesions are seen within the left supra-clavicular region, as described, likely metastatic lymph nodes, for histopathology correlation.

Best regards;
Dr. Mohammad Islam

Patient's Name: Nasira Ahmed Hassan Ahmed
Date: 2026-08-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST-CONTRAST MULTI-SLICE CT SCAN OF THE NECK, CHEST, ABDOMEN AND PELVIS

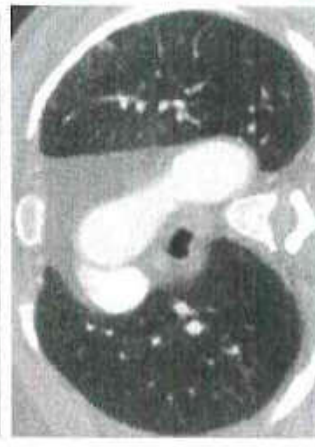
Known patient with left breast cancer. Compared to the previous study dated 21/6/2026:

I-NECK:

- The lower brain cuts taken are unremarkable.
- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal appearance of submandibular, parotid salivary glands.
- Normal size, enhancement & symmetrical appearance of thyroid glands.
- Normal course and caliber of the carotid and jugular vessels.
- Few subcentemetric bilateral upper deep cervical and submental LNs are noted; likely non-specific.
- No pathologically enlarged cervical lymph nodes.

II-CHEST:

- Clear left mastectomy bed by CT.
- Beam hardening artifact hinders proper assessment of the supraclavicular region. The suspected left supraclavicular LN now measures 1.7x1.6 cm compared to 2x0.8 cm in the previous study? for US correlation.
- Still noted patchy area of ground glass opacity in the anterior segment of left upper pulmonary lobe and linear atelectatic bands in the middle lobe and in the lingula.
- Still noted middle lobe patch of consolidation.
- No sizable suspicious pathological enlarged lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly. No pericardial effusion detected.
- Normal enhanced appearance of the thoracic aorta and the great vessels.



Patient's Name: Nasira Ahmed Hassan Ahmed
Date: 2026-08-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Right hypodense thyroid nodule for US correlation.

III-ABDOMEN AND PELVIS

- Average size of the liver showing homogenous low parenchymal attenuation likely fatty. No focal lesion or dilated biliary radicles.
- Patent homogenously enhanced portal vein and its main branches.
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No definite cysts, masses or back pressure changes.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, and IVC.
- Atherosclerotic changes of the aorta.
- The urinary bladder is empty.
- Bulky uterus for age.
- No suspicious abdominal or pelvic lymphadenopathy.
- No ascites.
- Average caliber of the bowel loops.
- Bone window reveals no suspicious lytic or sclerotic lesions.

OPINION :

- Follow up study of a patient with history of left breast cancer, current study reveals rather stationary findings.
- The previously suspected left supraclavicular LN is better to be assessed by US due to beam hardening artifact.

Shohenda. M. Taha, MD